



Questionnaire

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ORAL CANCER RISK QUESTIONNAIRE Documentation & Reference Guide

OVERVIEW

This is an interactive browser-based screening tool that evaluates a patient's risk for oral cancer based on their risk factors and symptoms. It is not a diagnostic tool — results are indicative only and direct the patient toward appropriate levels of clinical care.

The questionnaire has two main sections: Section A (Risk Factors) and Section B (Symptoms). Within Section B, three symptom questions unlock additional follow-up questions for differential diagnosis if the patient answers "Yes." After all questions are answered, the patient submits the form and receives one of five risk assessments.

SECTION A — RISK FACTORS (9 Questions)

These questions cover lifestyle habits, exposures, and medical history known to be associated with oral cancer.

A1. Do you currently smoke or use tobacco products (cigarettes, cigars, pipe, bidis)?

[Note shown: Includes past use within the last 10 years]

A2. Do you use smokeless tobacco — chewing tobacco, snuff, or gutkha?

A3. Do you chew betel nut (areca nut) or pan, with or without tobacco?

A4. Do you consume alcohol regularly?

[Note shown: More than 14 units/week (men) or 7 units/week (women)]

A5. Do you have a known HPV (Human Papillomavirus) infection, or have you been told you are at elevated risk?

[Note shown: HPV-16 is associated with oropharyngeal cancers]

A6. Are you frequently exposed to prolonged sunlight or UV radiation without lip protection?

[Note shown: Relevant for labial (lip) carcinoma]

A7. Do you have a family history of oral cancer or head and neck cancers?

A8. Do you have a personal history of prior oral lesions, dysplasia, or head and neck cancer?

A9. Are you immunocompromised or on long-term immunosuppressant medications?

[Note shown: E.g., organ transplant recipients, HIV/AIDS, long-term steroids]

Each question is answered with Yes or No.

A "Yes" to any A1–A9 counts as a CAUSE being present.

SECTION B — SYMPTOMS (9 Questions + Conditional Follow-Ups)

These questions cover signs and presentations associated with oral cancer. Three of the nine symptom questions (B1, B2, B3) will

unlock a set of differential diagnosis follow-up questions if the patient answers "Yes."

B1. Do you have any sores or ulcers in your mouth that have not healed within 3 weeks?

→ IF YES, the following 3 differential questions appear:

- D1. Did the sore appear within a few days of starting a new medication, having a dental procedure, or a possible allergic exposure?
[Note: Points toward drug reaction, contact stomatitis, or fixed drug eruption]
- D2. Does the ulcer recur periodically and heal completely on its own within 1–2 weeks each time?
[Note: A recurring, self-resolving pattern is characteristic of aphthous ulcers (canker sores)]
- D3. Did the sore begin as painful clusters of small blisters, possibly preceded by a tingling or burning sensation?
[Note: Suggests herpes simplex virus (HSV) or herpes zoster reactivation]

B2. Have you noticed any red (erythroplakia) or white (leukoplakia) patches inside your mouth that will not scrape off?

→ IF YES, the following 3 differential questions appear:

- D4. Can the white coating be easily wiped off with a piece of gauze or a cloth?
[Note: Removable white material strongly suggests oral candidiasis (thrush) rather than leukoplakia]
 - D5. Does the patch correspond exactly to an area of repeated local trauma – e.g., a sharp tooth edge, ill-fitting denture, or habitual cheek biting?
[Note: Traumatic keratosis can mimic early leukoplakia]
 - D6. Do you notice a lace-like or net-like white pattern (fine white lines) on your inner cheeks or gums rather than a solid patch?
[Note: Wickham's striae – a reticular pattern – are characteristic of oral lichen planus]
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B3. Do you have a lump, thickening, or persistent rough patch in your mouth, on your lip, or in your throat?

→ IF YES, the following 2 differential questions appear:

D7. Is the lump or swelling tender to touch, and did it appear shortly after a dental infection, sore throat, or upper respiratory illness?

[Note: Tender, post-infection swelling is typical of reactive lymphadenopathy, not malignancy]

D8. Has a clinician previously examined this lump and given a confirmed benign diagnosis?

[Note: E.g., fibroma, mucocele, salivary gland swelling, lipoma]

B4. Do you have persistent pain or soreness in your mouth, tongue, or throat without an obvious cause?

B5. Have you experienced unexplained difficulty chewing, swallowing, or moving your tongue or jaw?

[Note shown: Trismus or dysphagia without apparent dental cause]

B6. Have you had unexplained numbness or loss of feeling in any part of your mouth, face, or neck?

B7. Do you have a hoarse voice or persistent sore throat that has lasted more than 3 weeks?

B8. Have you noticed a swollen lymph node or lump in your neck that has persisted for more than 3 weeks?

B9. Have you had unexplained weight loss — without dieting — in the past 3 months?

Each question is answered with Yes or No.

A "Yes" to any B1–B9 counts as a SYMPTOM being present.

A "Yes" to any D1–D8 counts as a DIFFERENTIAL being present.

If a patient answers "No" to B1, B2, or B3, the differential questions for that symptom are hidden and any previous answers to those differentials are cleared.

HOW IT WORKS — LOGIC & SCORING

After the patient completes the form and clicks "Evaluate My Risk," the tool checks three binary flags:

CAUSE PRESENT → Any "Yes" answer in Section A (A1–A9)

SYMPTOM PRESENT → Any "Yes" answer in Section B (B1–B9)

DIFFERENTIAL → Any "Yes" answer among D1–D8, but only if the parent symptom (B1/B2/B3) was also answered "Yes" (i.e., the differential block was visible and engaged)

These three flags are combined to produce one of five outcomes:

ASSESSMENT RESULTS

RESULT 1: HIGH RISK — Seek Immediate Consultation

Trigger: CAUSE present AND SYMPTOM present (differential may or may not be present)

Title displayed:

"High Risk — Seek Immediate Consultation"

Message given to patient:

You have reported both significant risk factors and active symptoms associated with oral cancer. This combination warrants urgent clinical evaluation.

Please consult a dentist, oral surgeon, or oncologist as soon as possible — ideally within the next 1–2 weeks. Do not wait for symptoms to worsen.

Early-stage oral cancer is highly treatable. Prompt action is the most important thing you can do right now.

[If differentials are also present, the following note is added:]

Some of your symptoms may overlap with other conditions based on your follow-up answers. A clinician will rule out alternatives through examination and biopsy if required — this does not reduce the urgency of being seen.

Recommended actions listed:

- Book an urgent appointment with an oral health specialist
 - Bring a list of all medications, habits, and symptom duration
 - Do not attempt to self-treat or ignore lesions
-

RESULT 2: ELEVATED RISK — Caution Advised

Trigger: CAUSE present AND SYMPTOM not present

Title displayed:

"Elevated Risk — Caution Advised"

Message given to patient:

You have identified one or more significant risk factors for oral cancer but have not reported active symptoms at this time.

While the absence of symptoms is reassuring, your risk profile warrants attention. Schedule a routine oral cancer screening with your dentist if you have not had one in the past year.

Reducing or eliminating modifiable risks — especially tobacco, areca nut, and alcohol — can significantly lower your lifetime risk.

Recommended actions listed:

- Schedule an oral cancer screening examination
 - Discuss your risk factors openly with your dentist or GP
 - Consider cessation support if substance use is a factor
 - Perform monthly self-examinations and report any new changes lasting over 3 weeks
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RESULT 3: SYMPTOMS PRESENT — Consultation Recommended

Trigger: CAUSE not present AND SYMPTOM present AND DIFFERENTIAL not present

Title displayed:

"Symptoms Present — Consultation Recommended"

Message given to patient:

You have reported symptoms consistent with oral cancer, even without identified lifestyle risk factors. Oral cancer can occur in individuals with no obvious risk history.

We recommend consulting a dentist or oral health specialist to have your symptoms properly evaluated. Most oral lesions are benign, but persistent or unexplained changes always warrant professional assessment.

Recommended actions listed:

- Book a dental or oral medicine consultation
- Note the duration, location, and character of each symptom to share with your clinician
- If any symptom has persisted beyond 3 weeks, prioritise this appointment

RESULT 4: POSSIBLE ALTERNATE DIAGNOSIS — Consultation Still Advised

Trigger: CAUSE not present AND SYMPTOM present AND DIFFERENTIAL present

Title displayed:

"Possible Alternate Diagnosis — Consultation Still Advised"

Message given to patient:

Your symptom responses are consistent with oral cancer presentations, but your follow-up answers suggest these may also be explained by other, non-cancerous conditions — such as aphthous ulcers, oral candidiasis, lichen planus, traumatic lesions, or reactive lymphadenopathy.

This is important context, but it does not eliminate the need for professional evaluation. Differential diagnoses can only be

confirmed — and malignancy ruled out — through direct clinical examination and biopsy where indicated.

Recommended actions listed:

- Schedule a dental or oral medicine consultation
- Share the specific locations, duration, and any associated triggers with your clinician
- If a lesion has not resolved within 3 weeks of removing an apparent cause, escalate urgency

RESULT 5: NO IMMEDIATE CONCERNS IDENTIFIED

Trigger: CAUSE not present AND SYMPTOM not present

(differential is irrelevant in this case)

Title displayed:

"No Immediate Concerns Identified"

Message given to patient:

Based on your responses, you have not reported significant risk factors or active symptoms associated with oral cancer at this time.

This is encouraging — however, good oral health habits remain important for everyone.

Recommended actions listed:

- Attend regular dental check-ups at least once a year
- Maintain a healthy diet and limit alcohol consumption
- Avoid tobacco and areca nut products
- Perform periodic self-examinations of your mouth — look for any new patches, sores, or lumps
- Report any changes that persist beyond 3 weeks to your dentist

DECISION LOGIC SUMMARY

Cause?	Symptom?	Differential?	Result
Yes	Yes	any	1 - High Risk
Yes	No	n/a	2 - Caution
No	Yes	No	3 - Consult
No	Yes	Yes	4 - Alternate Dx
No	No	n/a	5 - No Concerns

Note: When both cause and symptom are present (Result 1), a differential also being present adds a contextual note to the message but does not change the outcome category — the urgency remains the same.

ADDITIONAL INTERFACE NOTES

Progress bar

A progress bar at the top of the form fills as the patient answers questions. It counts all currently visible questions (including any unlocked differential questions) and reflects the proportion answered.

Differential question visibility

- Differential questions for B1 appear only when B1 = Yes
- Differential questions for B2 appear only when B2 = Yes
- Differential questions for B3 appear only when B3 = Yes
- Switching a parent answer from Yes to No hides the block and clears all answers within it

Reset

A "Start Over" button appears beneath the result. It clears all answers, hides all differential blocks, removes the result card, and resets the progress bar to zero.

Disclaimer

The form displays a persistent disclaimer: "This tool is not a substitute for professional clinical evaluation. Always consult a qualified dentist or oncologist."